

# Surgical Block Scheduling Policy

Tallowbrook Health | Perioperative Services | 2025-08-01

Policy PS-210, revision 3, effective August 1, 2025. Approved by the perioperative committee and the medical executive committees of Tallowbrook Medical Center West and Hospital Fort Bend. Revision 3 lengthens the release window for orthopedics and spine and restates how utilization is measured.

## Purpose and scope

This policy governs access to operating room time at Medical Center West (10 operating rooms) and Hospital Fort Bend (6 operating rooms). The Surgery Center Grand Parkway is governed by its own board and scheduling rules and is not covered.

The aims are fair access for surgeons, predictable staffing for perioperative teams and full use of staffed operating room time.

## Definitions

Block: a recurring period of staffed operating room time assigned to a surgical service.

Released time: block time given back by the service before the release deadline.

Open time: unassigned staffed time plus released time, booked first come, first served.

Block utilization: in-room to out-of-room minutes used within the block, divided by allocated minutes minus minutes released on time.

## Allocation

Blocks are allocated to services, not to individual surgeons. Each service chief distributes the service's blocks among surgeons and informs Perioperative Services by the 15th of the prior month.

New services or surgeons request time through the Director of Perioperative Services. Requests are considered at the next quarterly review.

## Release rules

Unused block time must be released at least 72 hours before the day of surgery. Orthopedics and spine must release at least five business days ahead, because implant and equipment scheduling needs more notice.

Time not released by the deadline and not used counts against the service's utilization.

## Open and released time

Released and unassigned time is posted in the EHR surgery schedule and booked first come, first served by any credentialed surgeon. Add-on and urgent cases are scheduled by the OR charge nurse according to case urgency and are not charged to a block.

## **Quarterly review**

The perioperative committee reviews utilization by service each quarter. A service below 70% block utilization for two consecutive quarters is reviewed for reallocation. The committee may reduce the allocation, convert it to open time or keep it with a documented reason, such as a new surgeon building a practice.

## **Exceptions**

The chief medical officer of either hospital may suspend block rules during an emergency or a mass casualty event. Robotic room scheduling will be addressed in a future revision.